Encompass Health Rehabilitation Hospital of Las Vegas
1250 South Valley View Blvd Las Vegas, NV 89102-
Patient: LILORE, JENNIFER M
MRN: 875004778 Admit Date: 4/6/2026
FIN#: 501446081 Discharge Date:
DOB/Age/Gender: 8/17/1977 48 years Female Attending Phys: Hewett DO,Marie

Face Sheet

Encompass Health Rehabilitation Hospital of Las Vegas 7028778898

Encounter Information

FIN: 501446081
Admit Dt/Tm: 04/06/2026 17:01
Disch Dt/Tm:
Attend Physician: Hewett DO, Marie
Admit Physician: Hewett DO, Marie
PCP: Unavailable, Physician
Referring Physician: Kim D.O., Dong Hyun
Onset of Symptoms/Illness: (11) 03/30/2026

:

Third Party MRN:
Patient Type: Inpatient
Medical Service: OTH Other Conditions
Subspecialty:
Location: NURS LVNEU
Room/Bed: 138 / D
Admit Reason: NEURO CONDITIONS
Accommodation: General Semi Private
VIP Indicator:

Patient Information

Patient Name: LILORE, JENNIFER M
MRN: 875004778
Home Address: 4205 W TROPICANA AVE
LAS VEGAS, NV 89103
County: CLARK
Home Phone: (702)374-9469

Admin Sex: Female
DOB: 08/17/1977
Religion: Christian
Race: White
Email Address:
Mobile Phone:

Birth Sex: Female
Age: 48 Years

Guarantor Information

Guarantor Name: LILORE, JENNIFER M
Billing Address: 4205 W TROPICANA AVE
LAS VEGAS, NV 89103

Patient's Reltn: SELF
Home Phone: (702)374-9469
Mobile Phone:

Contact Information

Emergency Contact
Contact Name: LILORE, PATRICIA
Patient's Reltn: Parent
Home Phone: (702)493-0252
Mobile Phone:

Emergency Contact
Contact Name:
Patient's Reltn:
Home Phone: (702)493-0252
Mobile Phone:

Primary Insurance

Insurance Name: MCR Medicare
Financial Class: Medicare
Policy Nbr/Group Nbr: 4H66DD4KV61 /
MBI:

Claim Address: MCR A AND B
,
Authorization Number:

Secondary Insurance

Insurance Name: MDNV Medicaid Nevada
Financial Class: Medicaid Trad In State
Policy Nbr/Group Nbr: 00001731618 /
MBI:

Claim Address: NV MCD SLMB
,
Authorization Number:

Tertiary Insurance

Insurance Name:
Financial Class:
Policy Nbr/Group Nbr: /
MBI:

Claim Address:
,
Authorization Number:

Comments:
FROM SIENA
IM: PLATINUM

Printed by: Livingston, Marietta on 04/06/2026 17:05 to printer faceshe
Registration last updated by: Livingston, Marietta on 04/06/2026 17:05

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History and Physical

DOCUMENT NAME: History and Physical
SERVICE DATE/TIME: 4/7/2026 13:03 PDT
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Hewett DO,Marie (4/7/2026 10:06 PDT)
SIGN INFORMATION: Hewett DO,Marie (4/7/2026 16:26 PDT)

H&P REHAB

Patient: LILORE, JENNIFER M MRN: 875004778 FIN: 501446081
Age: 48 years Sex: Female DOB: 8/17/1977
Associated Diagnoses: None
Author: Hewett DO, Marie

Attending Provider: Hewett, Marie
Referring Physician: Kim, Dong
Primary Care Physician: Ferrer, Gerianne

History of Present Illness:
Jennifer M Lilore is a 48 year old female with PMH of HTN, asthma, seizure disorder, chronic pain syndrome with polyneuropathy and spinal cord stimulator, depression/anxiety, who was admitted to the emergency department from her skilled nursing facility on 03/30/2026 after she was last seen well at 4 PM and found unresponsive at 6 PM. Emergency medical services brought her to the hospital, where initial workup revealed a hemoglobin of 10.5, serum sodium of 145, and a urine toxicology screen positive for benzodiazepines. She was extremely somnolent and only arousable to sternal rub, requiring Narcan administration both at the skilled nursing facility and again by EMS en route to the hospital. Her hospital course was complicated by persistent altered mental status, garbled speech, repetitive words, twitching and jerking movements, suspected aspiration with acute hypoxia, and a negative EEG for seizure activity. Imaging studies including CT head without contrast and CT chest, abdomen, and pelvis with contrast showed no acute intracranial or cardiopulmonary pathology. She required bilateral wrist restraints and lower extremity splints for safety. Of note, she also has impaired mobility due to a recent fall and right lower extremity injury, which is immobilized in a cast. Patient is now admitted to Encompass acute inpatient rehab for comprehensive therapy program prior to safe discharge home.

Today, patient seen sitting up comfortably in bed. She had ortho follow up appointment this morning for her cast removal (now in CAM boot). She reports current pain level 9.5/10 though objectively appears comfortable during discussion. She has not used her prn dilaudid and pain currently managed with prn tylenol and scheduled gabapentin. She does typically take baclofen 20 mg tid at home, though she states she takes inconsistently. Discussed concern with baclofen withdrawal with sudden cessation, as well as possible lowering of seizure threshold. She is agreeable to resume at lower dose. No reported fevers, chills, chest pain, or shortness of breath.

Past Medical/ Surgical History:
as above

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History and Physical

Allergies (1) Active	Reaction
No Known Allergies	None Documented

Medications (12) Active
Scheduled: (3)
docusate sodium 100 mg Cap 100 mg 1 cap, Oral, BID, Start 04/06/26 21:00:00 PDT
gabapentin 400 mg Cap 800 mg 2 cap, Oral, q8hr, Start 04/06/26 22:00:00 PDT
levETIRAcetam 500 mg Tab 1,000 mg 2 tab, Oral, BID, Start 04/06/26 21:00:00 PDT

Continuous: (0)

PRN: (9)
acetaminophen 325 mg Tab 650 mg 2 tab, Oral, q4hr, Start 04/06/26 18:03:00 PDT
alum-mag hydroxide 1200-1200mg/30mL Oral Susp (30mL UD) 30 mL, Oral, q6hr, Start 04/06/26 18:03:00 PDT
hydrALAZINE 25 mg Tab 50 mg 2 tab, Oral, q6hr, Start 04/06/26 18:03:00 PDT
HYDROmorphone 2 mg Tab 4 mg 2 tab, Oral, q4hr, Start 04/06/26 18:01:00 PDT
naloxone 0.4 mg/mL Inj Sol 0.4 mg 1 mL, Intramuscular, q2min, Start 04/06/26 18:03:00 PDT
nitroglycerin 0.4 mg sublingual Tab 0.4 mg 1 tab, SL, q5min, Start 04/06/26 18:03:00 PDT
ondansetron 2 mg/mL Inj Sol 2 mL 4 mg 2 mL, IM, q4hr, Start 04/06/26 18:03:00 PDT
ondansetron 4 mg Tab 4 mg 1 tab, Oral, q4hr, Start 04/06/26 18:03:00 PDT
polyethylene glycol 3350 Oral Pwdr-Reconstit {17gm} (1EA UD) 17 gm 1 EA, Oral, Daily, Start 04/06/26 18:03:00 PDT

Family History: noncontributory

Social History:
Patient lives alone in a one-story home with no steps to enter. Prior to admission, patient was independent with ambulation, activities of daily living, and cognition. Currently, patient requires supervision for bed mobility and minimal assistance for transfers, ambulation, and basic ADLs, reflecting a significant decline in independence. Patient uses a walker for mobility. Substance use history, including alcohol, tobacco, and other substances, is unknown due to cognitive impairment. Patient's prior transportation abilities are not documented, but current functional decline and use of a walker suggest limited ability to travel independently.

Functional Assessment (current vs. prior level of function that preceded current medical/ surgical illness):
The patient was previously independent with ambulation, activities of daily living (ADLs), and cognition, but now requires supervision for bed mobility and minimal assistance for transfers, ambulation, and ADLs, reflecting a significant decline in functional status. This decline is associated with acute toxic-metabolic encephalopathy, recent fall with right lower extremity injury, and increased pain and fall risk.

Prior Level of Function: Ambulation: Independent ADLs: Independent Prior Cognitive Status: Independent Prior Physical Activity Level: Sedentary

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History and Physical

Current Level of Function: Bed Mobility: Supervision Transfers: Minimal Assistance Ambulation: Minimal Assistance Basic ADLs: Minimal Assistance Dressing: Minimal Assistance

Review of Systems:
Constitutional: negative
Vision: negative
ENMT: negative
Respiratory: negative
Cardiovascular: negative
GI: negative
GU: negative
Neurologic: negative
Musculoskeletal: negative
Skin: negative
Pain: negative
Psychiatric: negative

Data review:
Labs (Last four charted values)
Cr 0.72 (APR 06)

Diet: Diet -- 04/06/26 18:03:00 PDT, Texture: Level 7 - Regular, Liquid Consistency: Level 0 - Thin, Restrictions: No Restrictions

Physical Examination:

Vital Signs (last 24 hrs)	Last Charted
Temp Oral	97.8 DegF (APR 07 08:24)
Heart Rate Peripheral	99 bpm (APR 07 08:25)
Resp Rate	19 br/min (APR 07 08:25)
SBP	129 mmHg (APR 07 08:24)
DBP	89 mmHg (APR 07 08:24)
SpO2	96 % (APR 07 08:25)
BMI	36.3 (APR 06 20:00)

General: siting up in bed, NAD, cooperative to exam
HEENT: normocephalic, atraumatic, oropharynx clear
Chest: non-labored breathing

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History and Physical

Heart: regular rate
Abdomen: soft, NT, ND, + BS
Extremities: RLE in CAM boot
Skin: Warm and dry. no rashes or other markings on exposed areas.
Neurologic: awake, alert, oriented x3. CN II-XII grossly intact. 4/5 throughout on MMT. limited by pain. Sensation intact.

Wound Status
This data does not meet import criteria.

Assessment:
Metabolic encephalopathy
RLE injury/fx, now in CAM boot
Impaired gait, mobility, endurance
Requires assistance with ADLs
Obesity class II, BMI 36
HTN
Asthma
Seizure disorder
Chronic pain syndrome with polyneuropathy
Spinal cord stimulator
Depression/anxiety

Comprehensive Rehabilitation Plan
Rehab
- Initiate PT/OT/SLP within 24 hrs as delineated below.
- Precautions: Fall, RLE NWB/CAM boot
- DVT ppx: lovenox
- Diet -- 04/06/26 18:03:00 PDT, Texture: Level 7 - Regular, Liquid Consistency: Level 0 - Thin, Restrictions: No Restrictions
- Pain: APAP 650 mg q4h PRN. gabapentin 800 mg q8h resumed on admission. PO dilaudid 4 mg q4h prn
  * 4/7: add baclofen 5 mg q8h prn (home dose 20 mg tid)
- Bowel regimen: colace bid. miralax prn.

Additional medical issues:
- Baseline CBC and BMP
- Sz history: on keppra 1000 mg bid

Consultants:
- IM for management of medical comorbidities

Treatment Plan Anticipated
Required Disciplines: [PT, OT, SLP]
PT: ROM and stretching exercises. Bed Mobility, Transfers, Static/dynamic Balance, Ambulation with least restrictive adaptive device, Home management, Family Training. HEP.

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Admit Date: 4/6/2026
Discharge Date:
Attending Phys: Hewett DO,Marie

History and Physical

OT: BUE strengthening and stretching. ADL/iADL re-training with adaptive equipment PRN. Safety, Balance with self care, Gross/Fine Motor Movements, Modalities PRN, Splints PRN. Home management. HEP.
SLP: Cognitive-Linguistic Deficits.

Intensity of Services: [3 hours of therapy a day/5 days a week]
Estimated Length of Stay: [14 days]
Goals: Modified independent for self care, transfers and for ambulation.

Attestation:
Considering all of the information above, it is my best judgment that this patient requires intensive rehabilitation therapy as described above in an inpatient hospital environment due to the complexity of nursing, medical management, and rehabilitation needs, and can reasonably be expected to participate in and benefit from an inpatient stay with an interdisciplinary team approach to the delivery of rehabilitation care under the direction and supervision of a rehabilitation physician.

Electronically Authenticated on 04/07/2026 04:26 PM

Marie Hewett, DO

Transition of Care Communication

DOCUMENT NAME: Transition of Care Communication
SERVICE DATE/TIME: 4/15/2026 15:22 PDT
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Fernandez CM,Bryan (4/15/2026 15:23 PDT)
SIGN INFORMATION: Fernandez CM,Bryan (4/15/2026 15:23 PDT)

Contact Information
Name: LILORE, JENNIFER M
Address:
4205 W TROPICANA AVE
LAS VEGAS, NV 89103
Sex: Female
Date of Birth: 08/17/1977
Phone: (702)374-9469
MRN: 875004778
FIN: 501446081
Location: Encompass Health Rehabilitation Hospital of Las Vegas
Registration Date and Time: 04/06/2026 17:01 PDT
Primary Care Physician:
Ferrer FNP-C, MSN, Gerianne
Attending Physician:
Hewett DO, Marie
Language: English
Race:
White

Medications
Discharge Meds
baclofen 5 mg oral tablet, 5 mg= 1 tab, Oral, q8hr
diazePAM 5 mg oral tablet, 10 mg= 2 tab, Oral, BID, PRN
divalproex sodium 500 mg oral tablet, extended release, 1000 mg= 2 tab, Oral, QHS
gabapentin 400 mg oral capsule, 800 mg= 2 cap, Oral, q8hr
Imitrex 50 mg oral tablet, 100 mg= 2 tab, Oral, Daily, PRN
oxyCODONE 5 mg oral tablet, 5 mg= 1 tab, Oral, q6hr, PRN

Allergies
No Known Allergies

Therapy Current Level of Function
GG0130 Eating: Independent - 06 (04/15/26 12:00:00)
GG0130 Toileting Hygiene: Independent - 06 (04/15/26 09:00:00)
GG0130 Shower, Bathe Self: Independent - 06 (04/15/26 09:00:00)
GG0130 Upper Body Dressing: Independent - 06 (04/15/26 09:00:00)

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Discharge Date:
Attending Phys: Hewett DO,Marie

Transition of Care Communication

Ethnicity:
Not of Hispanic, Latino, or Spanish Orig

Providers

Admitting Physician -
Hewett DO, Marie

Attending Physician -
Hewett DO, Marie

Consulting Physician -
Cichon MD, Jamie
Huang DO, Ryan
Miranda MD, Yandy

Diagnoses

Anxiety disorder, unspecified, 04/14/2026
Headache, unspecified, 04/14/2026
Low back pain, unspecified, 04/14/2026
Polyneuropathy, unspecified, 04/14/2026

Discharge Orders

Order Name	Order Details	Electronically Signed By

Continuity of Care

Discharge Plan:
DME Provider:
SNF Provider:
Home Health Care Provider:
Outpatient Provider:
IRF Provider:
LTCH Provider:
Hospice:
Inpatient Hospice:
Additional Referrals:
Patient, Family Plan Concerns:

Advance Directive

Advance Directive Type:
Medical power of attorney name:
Surrogate Name:

Skin Integrity

Skin Integrity:Not intact
Incision/Wound Documentation
Right, Heel Other: wound - Inc/Wnd Dressing: Dry, Intact (04/15/26 12:30:00)
Right, Heel Other: wound - Inc/Wnd Photo Taken: No (04/15/26 12:30:00)
Right, Knee Abrasion - Inc/Wnd Cleansing/Irrigation: Cleansed with Wound Cleanser (04/11/26 14:58:00)
Right, Knee Abrasion - Inc/Wnd cm Length: 1 cm (04/11/26 14:58:00)
Right, Knee Abrasion - Inc/Wnd cm Width: 3 cm (04/11/26 14:58:00)
Right, Knee Abrasion - Inc/Wnd Comments: Seen by WPC; friction injury

GG0130 Lower Body Dressing: Independent - 06 (04/15/26 09:00:00)
Tub/Shower Transfer- OT: Ind (04/15/26 11:54:00)
GG0170 Toilet Transfer: Independent - 06 (04/15/26 09:00:00)
GG0170 Walk 10 Feet: Independent - 06 (04/14/26 14:30:00)
GG0170 Walk 50 Feet with Two Turns: Independent - 06 (04/14/26 14:30:00)
GG0170 Walk 150 Feet: Supervision or touching assistance - 04 (04/14/26 14:30:00)
GG0170 Walking 10 Feet Uneven Surfaces: Independent - 06 (04/14/26 14:30:00)
GG0170 1 Step (Curb): Independent - 06 (04/14/26 14:30:00)
GG0170 4 Steps: Independent - 06 (04/14/26 14:30:00)
GG0170 12 Steps: Supervision or touching assistance - 04 (04/14/26 14:30:00)
GG0170 Wheel 50 Feet with Two Turns: Independent - 06 (04/15/26 10:00:00)
GG0170 Wheel 150 feet: Independent - 06 (04/15/26 10:00:00)
Comprehension Mode Functional Status: Auditory (04/06/26 20:00:00)
Expression Mode Functional Status: Vocal (04/06/26 20:00:00)
BB0700 Expression of Ideas and Wants: Expresses complex messages without difficulty and with speech that is clear and easy to understand - 4 (04/15/26 12:30:00)
BB0800 Understanding Verbal Content: Understands - Clear comprehension without cues or repetitions - 4 (04/15/26 12:30:00)
Appetite: Good (04/15/26 12:30:00)
H0400 Bowel Management: Continent (04/15/26 12:30:00)
H0350 Bladder Management: Continent (04/15/26 12:30:00)

Risk Factors

High fall risk

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Transition of Care Communication

POA. (04/11/26 14:58:00)
Right, Knee Abrasion - Inc/Wnd Description/Assess: Epithelialized (04/11/26 14:58:00)
Right, Knee Abrasion - Inc/Wnd Drainage: Scant, Serosanguineous (04/11/26 14:58:00)
Right, Knee Abrasion - Inc/Wnd Dressing: Foam (04/15/26 12:30:00)
Right, Knee Abrasion - Inc/Wnd Pain Scale: 0 (04/11/26 14:58:00)
Right, Knee Abrasion - Inc/Wnd Photo Taken: No (04/15/26 12:30:00)
Right, Knee Abrasion - Inc/Wnd Signs of Infection: None (04/11/26 14:58:00)
Right, Lateral, Ankle Surgical incision - Inc/Wnd Comments: Seen by wound APRN Capistrano, C.; see provider notes for details (04/09/26 11:00:00)
Right, Lateral, Ankle Surgical incision - Inc/Wnd Dressing: Dry, Intact (04/15/26 12:30:00)
Right, Lateral, Ankle Surgical incision - Inc/Wnd Photo Taken: No (04/15/26 12:30:00)
Right, Medial, Ankle Traumatic - Inc/Wnd Base Tissue: Granulation (04/08/26 16:45:00)
Right, Medial, Ankle Traumatic - Inc/Wnd Cleansing/Irrigation: Cleansed with Wound Cleanser (04/08/26 16:45:00)
Right, Medial, Ankle Traumatic - Inc/Wnd cm Length: 2.5 cm (04/08/26 16:45:00)
Right, Medial, Ankle Traumatic - Inc/Wnd cm Width: 2.5 cm (04/08/26 16:45:00)
Right, Medial, Ankle Traumatic - Inc/Wnd Comments: Seen by wound APRN Capistrano, C.; see provider notes for details (04/09/26 11:00:00)
Right, Medial, Ankle Traumatic - Inc/Wnd Description/Assess: Granulated, Necrotic tissue. Slough (04/08/26 16:45:00)
Right, Medial, Ankle Traumatic - Inc/Wnd Drainage: Moderate, Serous (04/08/26 16:45:00)
Right, Medial, Ankle Traumatic - Inc/Wnd Dressing: Dry, Intact (04/15/26 12:30:00)
Right, Medial, Ankle Traumatic - Inc/Wnd Pain Scale: 6 (04/08/26 16:45:00)
Right, Medial, Ankle Traumatic - Inc/Wnd Photo Taken: No (04/15/26 12:30:00)
Right, Medial, Ankle Traumatic - Inc/Wnd Signs of Infection: None (04/08/26 16:45:00)
Right, Medial, Ankle Traumatic - Inc/Wnd Surrounding Skin: Erythema (04/08/26 16:45:00)
Right, Medial, Ankle Traumatic - Inc/Wnd Surrounding Tissue: Maceration (04/08/26 16:45:00)
Right, Medial, Ankle Traumatic - Inc/Wnd Wound Edges: Attached (04/08/26 16:45:00)

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Home Health Referral

DOCUMENT NAME: HH Referral Progress Note-Physician
SERVICE DATE/TIME: 4/16/2026 10:39 PDT
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Hewett DO,Marie (4/16/2026 10:39 PDT)
SIGN INFORMATION: Hewett DO,Marie (4/16/2026 10:39 PDT)

Home Health Referral

Patient: LILORE, JENNIFER M MRN: 875004778 FIN: 501446081
Age: 48 years Sex: Female DOB: 8/17/1977
Associated Diagnoses: None
Author: Hewett DO, Marie

HOME HEALTH CERTIFICATION and FACE to FACE ENCOUNTER ADDENDUM

A FACE TO FACE ENCOUNTER indicating the reason(s) this patient requires home health services was performed on DATE: _ 04/16/2026
Diagnoses and Clinical condition(s) contributing to the need for Home Health Care (specify):
Metabolic encephalopathy
RLE injury/fx, now in CAM boot
Impaired gait, mobility, endurance
Requires assistance with ADLs
Obesity class II, BMI 36
HTN
Asthma
Seizure disorder
Chronic pain syndrome with polyneuropathy
Spinal cord stimulator
Depression/anxiety

The patient is confined to the home for the following reason(s):
[] Unable to ambulate
[] Unable to ambulate > ___ feet without rest periods
[X] Requires assistive device: wheelchair, walker and/or assistance of another person to leave home
[] Requires the use of special transportation
[] Confusion/Cognitive limitations make it unsafe for patient to leave home
[] Pain interferes with outside mobility
[] SOB with exertion/activity, requires rest periods
[] Medically restricted to home (reason if not specified above): []

Prescriptive needs:
[X] Medication Management
[] Pain assessment and management
[] Disease Process Education
[X] Gait Training
[X] ADL training
[X] Fall Risk/Balance Assessment and training
[X] Strength/ Range of motion training
[] Speech training

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Home Health Referral

[] Cognitive retraining
[] Dysphagia training
[] Safety Assessment and training
[] Wound Care: []
[] other: []

HOME HEALTH REFERRAL AND ORDERS:

Discipline-specific frequency and duration of service recommendations may be further specified by the physician responsible for the home care plan of care.

[X] Skilled Nursing for: [X] med mgmt. [X] teaching and training [] Wound care [] Other:
[X] Occupational Therapy for: [X] ADLs/IADLs [X] therapeutic ex. / activity [] Other:
[X] Physical Therapy for: [X] gait/balance [X] therapeutic ex. / activity [] Other:
[] Speech Therapy for: [] Dysphagia [] Cog [] Speech / Language [] Other:
[X] Home Health Aide for:
[X] bathing / personal care as indicated by the comprehensive assessment with input from the interdisciplinary team and community physician.
[] Social Worker 1-2 visits for:
[] assessment, education and/or action on community resources, and counseling as indicated

This patient is currently under my care, and I have initiated the establishment of a plan of care for home health services following the face-to-face encounter on the date noted above. I certify that this patient is confined to his/her home and needs intermittent skilled services (nursing care, physical therapy and/or speech therapy or continues to need occupational therapy) as indicated in the above summary of impaired body structures that supports the homebound status and need for skilled care. This patient will be followed by their PCP who will be responsible for further review of the plan of care and recertification as applicable.

Electronically Authenticated on 04/16/2026 10:39 AM

Marie Hewett, DO